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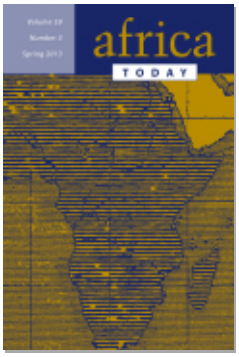

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Ghana has lost a considerable number of doctors and nurses to migration and has a critical shortage of health professionals. Health professionals are needed to bring healthcare to the people and to help achieve the Millennium Development Goals.

Back to My Roots: A Study of "Returning" Emigrated Health Professionals in the Greater Accra Region of Ghana

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The migration of health professionals from low-income to high-income countries affects the smooth functioning of health systems. Nevertheless, health professionals who return home help strengthen the health sector, leading to national development. This exploratory study focuses on factors influencing the return migration of health professionals and the benefits thereof to Ghana. The qualitative method was used to gather information from sixty respondents who consented to participate. The pull factor for migration was high income, while for return migration, the completion of projects and imparting knowledge to young health professionals were the dominating pull factors. These health professionals come with skills and expertise and create employment opportunities. Challenges faced were bureaucracy and constraints on support from government agencies.

The migration of health professionals from sub-Saharan African countries has significantly increased since 2000, with most countries becoming sources of migrants. In addition, emerging health threats on the African continent have affected the migration of these health professionals (Syred 2011; Naicker et al. 2009). In recognizing the demerit to the countries of origin, Lowell and Findlay (2001) have submitted that, "The emigration of skilled human capital from one country to the other in search of better returns of one's knowledge, skills, qualifications, competencies and make a better living"

is depleting the human capital resource of many low-income countries. In Ghana anecdotal information is that there is a continuous outflow of doctors and nurses to high-income countries after their training, for better conditions of life and service. Some go for further training and the pursuit of higher education. For example, in 2003, 850 health and medical personnel migrated to the United Kingdom (Naicker et al. 2009). Despite these numbers, return migration is a growing phenomenon, which has not received much attention, especially in sub Saharan Africa and, for that matter, Ghana. As of 2007, 968 return health professionals were working in the Greater Accra Region (Asabir 2008). This is a human-resource benefit to the health sector and indicates that return migration has to be given attention so that strategies and policies influencing it can be strengthened.

The Ghanaian health sector faces challenges with retaining health professionals, particularly doctors and nurses, as migration threatens the availability and quality of service delivery in the country. Ghana has lost a considerable number of doctors and nurses to migration and has a critical shortage of health professionals (World Health Organization 2006). Health professionals are needed to bring healthcare to the people and to help achieve the Millennium Development Goals. The migration of health professionals partly explains this shortage. The potential for return migration needs to be given full attention as a strategy to strengthen health systems: "properly managed return migration of health professionals could be an important tool to strengthening the health systems in their countries of origin" (Haour-Knipe and Davies 2008). Return migration has benefits and effects that extend beyond the individual returnee to the state, are envisaged to be lucrative for the country of origin and a productive and positive development strategy (Co, Gang, and Yun 2000). Recent observations and anecdotal reports show that more and more health professionals are returning to their home country, including Ghana.

Researchers in the area of migration have indicated that the successful return and reintegration of return migrants in their countries of origin have helped improve the human, financial, and social capital of these countries, thereby contributing to development (Naicker et al. 2009). In Ghana, however, there is little documented information on return health professionals (Delango Dovlo and Nyonator 1999). There is therefore a need to understand the factors influencing the decision to return so as to fashion strategies that will enable the health sector to tap the human, financial, and social capital that return migrants bring and thereby to improve the health sector and eventually the health of the people.

Objective of the Study

Main Objective

The aim of the study was to explore and investigate the factors influencing the return of emigrated health professionals.

Specific Objectives

The study specifically examined:

- Push and pull factors of the migration of health professionals
- Factors influencing return migration (pull-push) of health professionals
- Benefits of the return migration of health professionals to the health sector
- Challenges return migrants face upon return and strategies to facilitate and encourage return

Methods

Study Design

The study took an exploratory case-study approach. This was useful as return migration is a new phenomenon in Ghana, and little is known about it. This type of method sheds light on reasons why the return-migration trend is being noticed and its underlying processes.

Study Approach

Qualitative data were collected and analyzed to address the research objectives. Participants were purposively selected using the snowball technique within a period of six weeks. Sample size calculation for qualitative data was not as formulaic as is done for quantitative data. This does not mean that sample-size issues were unimportant in the data-collection process. Recent work discusses in great detail the issue of qualitative sampling schemes and the need to ensure that qualitative sample sizes should not be so small that it is difficult to achieve data saturation, theoretical saturation, or information redundancy; at the same time, the sample should avoid being so large that it inhibits deep, case-oriented analysis (Onwuegbuzie and Collins 2007; Onwuegbuzie and Leech 2007). In effect, a total sample of sixty out of the seventy-two health personnel identified took part in the study. These consisted of twenty-two doctors and thirty-eight nurses who had returned to

Ghana between 2000 and 2010 and were still engaged, one way or another, in the health sector.

Study Area

The study was carried out in the Greater Accra Region of Ghana among health professionals who had returned to Ghana within the previous ten years. This region was selected because it is the nation's capital and the seat of government, where most people returning from all parts of the world reside. In addition, it boasts of many hospitals, clinics, and diagnostic and teaching institutions, where most of these returnees are working. The choice of the study area was informed by the view that most returning health professionals had to have made Accra one of their first points of call in the process of return (Bangdiwala et al. 2010). Having worked in high-income countries, most professionals prefer working in similar conditions on their return. Hence, Accra becomes their first choice of place for work.

Analysis

The data were analyzed and coded under the following themes:

- Push and pull factors of migration of health professionals
- Pull and push factors of migration
- Benefits of return migration to the home economy, specifically the health sector
- Challenges faced by returnees and strategies to facilitate and encourage return migration

Definition of Terms

Push Factors: These are influences that arise from within the source country and facilitate a potential migrant's decision to leave (Delanyo Dovlo 2003; Dela Dovlo and Martineau 2004).

Pull Factors: These are actions and omissions of recipient countries that create the demand for or encourage potential migrants to leave (Delanyo Dovlo 2003; Dela Dovlo and Martineau 2004).

Findings

Migration of Health Professionals

A majority of the health professionals identify major push factors as limited opportunities for personal development, low income, minimal job

satisfaction, and poor working conditions. In contrast, political instability and the quest to join a partner abroad were not likely push factors. A majority of health professionals were generally not influenced by push factors, and a few of them mentioned lack of appreciation for work despite working hard.

Some pull factors were equally influential in driving the health workers away. There is a strong relationship between some pull factors and push factors, depending on local situations. For example, income was the most prominent pull factor, since almost all the respondents were generally influenced by higher income. Other important pull factors were a better standard of living, better working conditions, a better work environment, and the availability of working logistics. In addition, the desire to save money for projects and make use of opportunities to further individuals' education were identified. A female health professional said: "My basic salary was very small. I come from a poor background and I had to do something. I had to travel if I was to make any headway in life." A doctor stated:

> In the 1980s, things were tough, and my family was struggling. I had to work, but I saw that this was only possible if I traveled. I had contacts in the USA who told me that I could earn some money and at the same time continue my education.

This is evidence that push and pull factors are closely related: they hardly work in isolation. Nevertheless, these factors pertained as reasons for the continued stay of health workers abroad. This is consistent with what was found by Naicker et al. (2009).

Return Migration

A number of reasons account for why health professionals would come back to Ghana. A significant number of health professionals were motivated to return home because they had finished with building projects they were undertaking while working abroad. A doctor put it this way: "I have been able to complete my building and opened a clinic. I see no reason why I should continue to stay abroad."

Additionally, many people considered the motivation to help the healthcare system and the opportunity to impart knowledge to others in Ghana. Such respondents perceived their skills to be a wealth of experience and a pull factor to come to Ghana. Nevertheless, the issue of improvement in the health sector and in the standard of living in Ghana generally was considered a reason to return by very few respondents.

Benefits of Return Migration

The dominant skills acquired by the health workers were expertise in in-vitro fertilization, neonatal abnormalities, dialysis, gastrointestinal surgery, hematology, pediatrics, obstetrics, gynecology, anesthesia, ear, nose, throat, postpartum, geriatrics, cardiothoracic, oncology, dietary, and instrumentation. The health professionals seem to place a lot of emphasis on skills they had acquired through training or practice. These skills were more medical, compared to skills in management and teaching, which were mentioned by a few of them. To explain the importance of such skills, one midwife said:

> I can now arrest very complicated issues such as postpartum bleeding, and that is very important. For me, timeliness in solving emergency situations is very important and indeed necessary to save lives.

According to another respondent, these experiences and skills are reflected in the work they do when they return. This is indicated in a nurse's statement:

> We acquire some new experiences and skills which we bring back to enrich the service in Ghana. On the wards, you see that the returnee is clearly different, punctual, courteous, and very professional in his or her approach. We are passionate about medico-legal issues and how in this part of the world we do take things for granted.

A doctor stated:

> Now that I am here and working, I have solved a human-resource factor by one health professional, and I hope many will return to help with the human-resource problem.

For many respondents, the decision to return to Ghana has paid off. Some expressed happiness because, while they have reunited with family, they have become able to offer the needed support to family relations. A nurse said: "I am so happy to see my family and be reunited with them. It has been such a long time."

A significant number of respondents have established clinics, diagnostic centers, laboratories, and daycare centers and invested in business ventures and the stock exchange, and they therefore saw their contribution in the fact that they offered employment to people and at the same time contributed to the economy of the country through the taxes they were paying on their businesses and investments. Others saw their return worthwhile because they occupied bigger positions now in Ghana. Some had secured jobs as lecturers in universities and nursing institutions. Generally, they were

happy and had no regrets for returning to Ghana, as noted by a doctor: "I am happy that I came back because I now occupy a bigger platform."

Challenges of Return Migration

Healthcare professionals experienced challenges upon their return to Ghana. Some of these challenges were more institutional, such as the level of support received from governmental agencies in Ghana, governmental agencies abroad, and the health sector in Ghana. Other challenges were the constraints associated with movement from their country of residence (their host country) to Ghana and the long wait to secure appointments for interviews, receive a response, and secure an appointment letter for work. A nurse respondent said: "By the time one is done going through an interview and getting an appointment letter, your colleagues who went to the private sector would have moved ahead and already settled in."

Other challenges occur at the facility level, and they include interpersonal challenges, as mentioned by some respondents. One said:

> When you return, of course your colleagues would also have moved on to higher positions but they still feel intimidated by you and would want to frustrate you. Some administrations would even have you start from where you left off—which to me is a discouragement to return.

Another said:

> Sometimes at work, you may get an understanding manager; you may also come across the other one, who will let you feel that, when things were difficult when you left. Now that you have come, they make things difficult for you and make you feel alone.

Discussion

Push-Pull Factors as Motives of Health Professional Migration

The study found that factors such as limited opportunities for personal development, a limited ability to acquire one's own house on one's current salary, a limited opportunity to acquire one's own vehicle and some basic necessities, minimal job satisfaction, low income, and poor working conditions were responsible for pushing health professionals out of Ghana to seek better conditions of service. These findings support Best and Idyorough's work (2003), which suggests that individuals migrate in search of food and

security. The findings are consistent with reasons given by Naicker et al. (2009) as being responsible for the migration of health professionals.

For many health workers, owning a building and having a vehicle and a reasonable amount of money in a bank account are pertinent to continually staying on a job. This opinion is motivated by the perceived status of the profession of medical practice or health personnel. This plausible explanation may seem more practical for three reasons. The first is that the most touted migration in Ghana is that of health workers. The second reason is that a majority of health personnel leave within the first few years of practice without giving themselves ample time to grow and develop to a position where they can earn more rewards for their work. Third, other professions, though important to the development of the nation and requiring high qualification (just like nurses and doctors), do not see most of their professionals leave the country as often. The third reason is stated against the backdrop of the fact that a significant proportion of the work force in Ghana (including health personnel) are low- or middle-income earners, suggesting that the incentive to migrate is more social.

An important pull factor other than remuneration was the opportunity for personal development, which was limited in Ghana, as asserted by the health professionals. Humans by nature would want to be progressive in life, in terms of higher education, status, and the development of skills. That is why Hagopian et al. (2004) suggested that large numbers of African-trained physicians leave home upon completion of their medical-school training to further their education in various specialties in their profession. Even when there is not much money accrued to them and personal-enhancement structures exist and are accessible to them, health workers would feel progressive and stay back in the country. Some workers work hard just to make enough money to achieve these goals. This highlights the assertion by Landon et al. (2004) that improving access to training is an important element of improving retention. It suffices that policymakers can create the enabling environment that would make health professionals feel worthy by working in their home countries.

Poor working conditions were a push factor identified by the study. Poor working conditions encompass the safety and healthfulness of the workplace, including the physical work environment and the procedures followed in performing work. This is clearly a demotivating factor for hard work and lowers productivity—which contradicts workers' goal to live to their highest potential. The issue of poor working conditions was a factor that pushed health professionals away from Ghana. Doctors and nurses perform best when the hospitals they work in are equipped with modern diagnostic equipment and good furnishings and lighting systems. Such attractive work conditions in the advanced countries pull health personnel from developing nations like Ghana.

In contrast, the most compelling pull factor for migration was income. Given the above unattractive condition in the home country, it was easy to see the relative opportunities that other countries presented. Indeed,

opportunities are seen only in the light of present circumstances. Income was the most attractive consideration for travel because most of the aforementioned reasons are hinged on remuneration. Responses showed that the economic motivation to travel abroad took its inspiration from the poor financial background of some health professionals.

The increase in migration of health professionals has come about as a consequence of increasing integration of world economies, the changing needs of host and origin countries, and people's different needs (Nyberg–Sørensen, Hear, and Engberg–Pedersen 2003), facilitated by high technology and reliable transportation systems (Haour-Knipe and Davies 2008).

Factors (Pull-Push) Influencing Health Professionals to Return Home

The current study found that pull rather than push factors played a dominant role in return migration. Given the responses on satisfaction with return migration, it was obvious that most of the health professionals have returned for good. As noted by Cassarino (2004), it is only logical that since most health professionals migrated as a result of economic benefit in the receiving country, they would return once they felt they had achieved their aim or goal and were ready to reintegrate into their home country.

The motivation to impart knowledge arises from a sense of fulfillment on seeing that one's efforts are appreciated. With an improved skill set, migrant health workers upon their return tend to occupy bigger positions (Haour-Knipe 2009) and are in a better position to share experiences gained from a more advanced healthcare system. Closely linked to this is the motivation to help improve the healthcare system in Ghana. Returnees' willingness to work in Ghana and earn lower salaries to improve the health sector supports Haour-Knipe and Davies (2008), who mention that returned migrants bring with them new and modern ideas, values, skills, ambitions, and savings, all of which contribute to nation building.

Other factors that receive partial consideration to stimulate return are improvements in remuneration in the home country, in the health sector generally, and in the general standard of living. Improvement in remuneration, by virtue of the fact that it received only partial consideration by health professionals, implies that the health workforce is not necessarily motivated by income to return home, since remuneration in the home country is still considerably lower than it is abroad (Vujicic et al. 2004), but it may be motivated by a desire to acquire landed property and other businesses.

Benefits of Return Migration to the Country of Origin

The dominant benefit acquired by health personnel was in the area of skill acquisition. Many acquired skills in in-vitro fertilization, neonatal abnormalities, dialysis, gastrointestinal surgery, hematology, pediatrics, obstetrics, gynecology, anesthesia, ear, nose, throat, postpartum, geriatrics, cardiothoracic, oncology, dietary, instrumentation, and other skills in

management and teaching. These skills are important to the development of the health sector in Ghana. Their *import* supports the proposition by Straubhaar (2000) that countries of origin stand a chance of benefiting from the transfer of knowledge. It confirms the findings of Lowell and Findley (2001) that return migration helps improve healthcare through human, financial, and social capital.

Among other benefits to society in more economic terms was the creation of employment through the setting up of clinics and diagnostic centers for health workers in the home country. This reduced unemployment marginally, providing a source of income, not only to the employed, but to the families of those employed, making them better off (Petras and Kousis 1988), as well as contributing to the government treasury by way of taxation. This is a financial benefit to the country, as the employee and employer both get taxed on their income.

Another social benefit to returning home was in the reunion of families. A section of the respondents considered their family in their decision to return home. In fact, when asked about the benefit of their return, some, rather than citing the benefit inherent in their skill set, posited that they were happy to see and be with their families. The family benefits from the return of a member both in financial and social (bonding) terms.

Challenges Return Migrants Face upon Return and Strategies to Facilitate and Encourage Return

The lack of support services in host and home countries and in the health sector of home countries was the most mentioned problem. The bureaucracy and unwelcoming attitude of the Ministry of Health, the Ghana Health Service, and the Nurses and Midwives Council were specific examples cited. This is evidence that the country has not done much on its own to encourage returns. Some returnees submitted that they would want to go back to their host countries. This practice, if continued, gives credence to Liese and Dussault (2004), who predict that it will be difficult for African countries like Ghana to achieve the health-related Millennium Development Goals by the stipulated time of 2015, with the reason being that there is in the health sector a lack of a stable human-resource base that in fact is both institutional and human related.

Conclusion

The migration of health professionals from any country, especially sub-Saharan, and to be particular, Ghana, has affected the health system negatively, but return migration, even though negative, could bring skills needed to improve healthcare by developing new ways to encourage the return of health professionals. This study has shown that the reasons that make health professionals migrate out are not different from the ones that make

them return, except that economic push factors weigh more in the decision to migrate, whereas pull factors, such as the desire to impart knowledge and skill, complete a project, be with a spouse, and so forth dominate the return. These push-pull factors include poor or better compensation, security and education that are in the host countries, and the motivations to return centered on the improved status of the home country and the desire to be of service to it. The health care returnees in this study were middle-aged and had acquired human, social, and financial capital that they had brought to their home country for development amid a challenging terrain. These returnees are in Ghana for various reasons, mostly not for economic gain, but for service to their homeland. Their migration experiences have generally been positive, leading to the decision to return (the return of innovation) and the contribution they are making toward national development through the human, social, and financial benefits they brought along with them. Though much on the international front is being done about strategies to facilitate and encourage more returns and slow migration, these returnees do not seem to appreciate the efforts made by Ghana, if there are any efforts in the first place.

Problems of human resources will continue to confront the health sector except where lasting and effective solutions are brought to bear on how return migration can be beneficial. Since migration cannot be stopped, return migration should be encouraged so that there is a win-win situation for sending and receiving countries. Return migration is gaining popularity in Ghana, with more migrants coming back home. It therefore becomes imperative for the government and the health sector to look at this phenomenon and fashion ways of welcoming these migrants to help build the health sector and the nation, as research has shown that return migration indeed brings about national development. Returning to a changed country where social relations, political structures, economic conditions, and educational structures have improved, and not what they used to be, is attractive and influential (De Haas, Fokkema, and Fihri 2009). It is in this direction that the continuous improvement of health professionals' economic situations and their working conditions will go a long way to reduce the emigration of health professionals in Ghana.

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